

RehletShifaa Users, Roles and Access Guide

A plain-language guide for platform owners, provider organizations, clinic managers, RehletShifaa staff and testers

Edition 2 | 25 September 2026 | Replaces Edition 1 of the same date

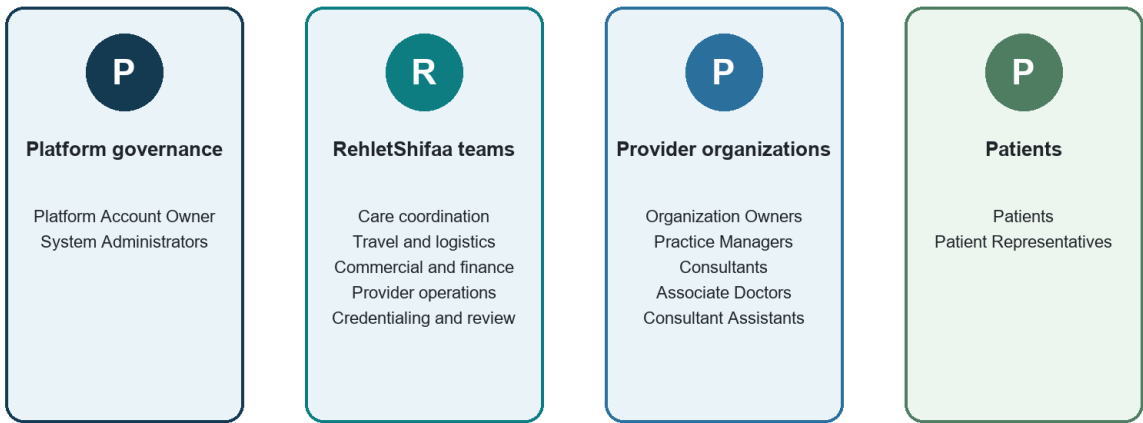
Status: planned, not yet available

This guide describes how access will work in the first production release of RehletShifaa. That release is not built yet.

Today's development system still works the old way in several important places. Section 2 lists the differences.

Testers: the handbook in Section 12 is the acceptance test for the first production release. Many of its tests fail on today's system by design.

Who uses RehletShifaa



One platform, four user populations, clear boundaries

Document owner: RehletShifaa. Source: Customer Governance Target Architecture, revision 3 (final), decisions D1 to D35.

1 How to use this guide

The model is simple. A role says what a person may do. A scope says where the role applies. A person gets access only when everything that applies is active at the same time: the person, their membership or coverage, the organization, the clinic, the role, and any professional qualification the work needs.

Reader paths

Reader	Start here	Then read
Provider owner or buyer	Sections 3, 6 and 9	Worked example (Section 13)
Clinic manager	Section 6 (organization and clinics)	Sections 8, 9 and 10
RehletShifaa manager	Sections 4 and 5	Section 10 (joining and leaving)
Tester	Section 2, then Section 12	Sections 8 to 11 for expected results
Implementation team	Section 2 and Section 14	All role profiles

How firm is each statement?

Almost everything in this guide is an agreed rule. Where a statement rests on a sensible default rather than an explicit business decision, or has not been decided at all, it is labelled:

Label	Meaning
Agreed rule	Decided in the architecture. Unlabelled statements are agreed rules.
Adopted default	The architecture's recommended default. It stands unless the business explicitly changes it.
Not yet decided	The architecture does not answer this. Do not build or test an answer until it is decided. See Section 14.
Names pending	English and Arabic role and clinic names are still in native-language review. Wording may change; meaning will not.

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2 What exists today and what is planned

The first production release (called P1 in the architecture) is delivered as one package. None of its parts goes live on its own, and there is no rollback to the old behaviour once it is live. Until P1 is built, the development system behaves as shown in the middle column.

Area	Today's development system	First production release
Administrator and auditor access to medical data	The old administrator and auditor roles can open the medical documents of any case.	System Administrator and Compliance Auditor have no access to patient cases or documents.
Strong sign-in (second factor)	Not configured. Password only.	Required for the owner, all internal staff, provider managers and clinicians.
Clinics and branches	Do not exist. The organization is the only unit; locations are free text.	Each organization has clinics with their own status, services, prices and schedules.
Inviting a person who already has an account	The person is attached to the organization silently, by email.	The person receives an offer and must accept. Nothing is active before acceptance.
Consultant or Associate Doctor level	One platform-wide type, sometimes a default value rather than a real declaration.	Declared level plus a separate credential decision for each organization.
Internal staff jobs	Granted as sign-in system roles.	Granted as RehletShifaa assignments: one job plus optional extra responsibilities.
Platform owner and administrators	A setup account holds broad legacy roles.	An owner relationship plus approved administrator appointments.
Staff joining and leaving	Separate manual commands.	Guided invitation, activation at first strong sign-in, and orchestrated offboarding.
RehletShifaa coverage of a provider	A provider owner can currently remove RehletShifaa's credential reviewer.	Providers cannot see, change or remove RehletShifaa coverage.

Evidence: architecture review sections 2 and 14A.1, checked against the code at commit e4c7552 on 25 September 2026.

What this means for testers

Run the Section 12 handbook only against a build that contains the whole first production release. On today's system, a failed test in Section 12 is expected. Record it as "not yet built", not as a defect.

3 The model in one page

Seven facts every reader should know

1. One human has one sign-in identity across the whole platform.
2. A provider organization is the legal or contracting party. It is the boundary between providers.
3. A clinic, branch, hospital or virtual-care site is a facility inside exactly one provider organization. The screens call these "Clinics & branches".
4. A provider professional may belong to several organizations and work at several clinics in each one.
5. The same person may hold different compatible roles in different organizations or clinics.
6. Ending one role, clinic link or membership never switches off access anywhere else.
7. Switching off a person's sign-in completely is reserved for account compromise or an authorized platform-wide action.

The provider hierarchy

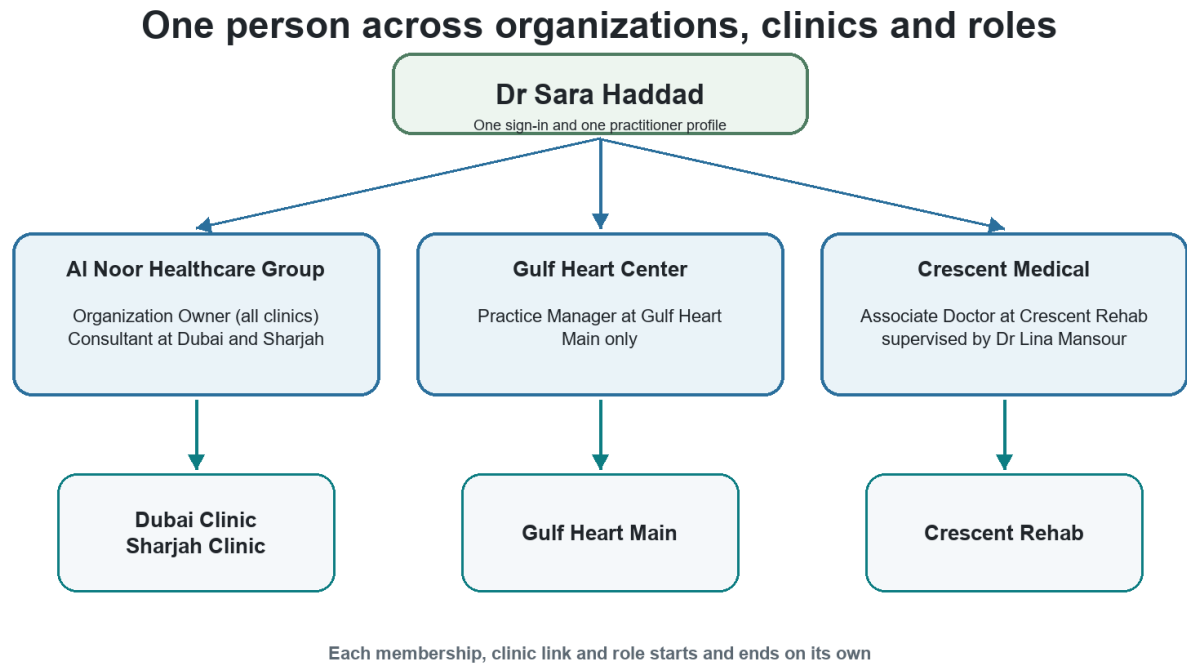


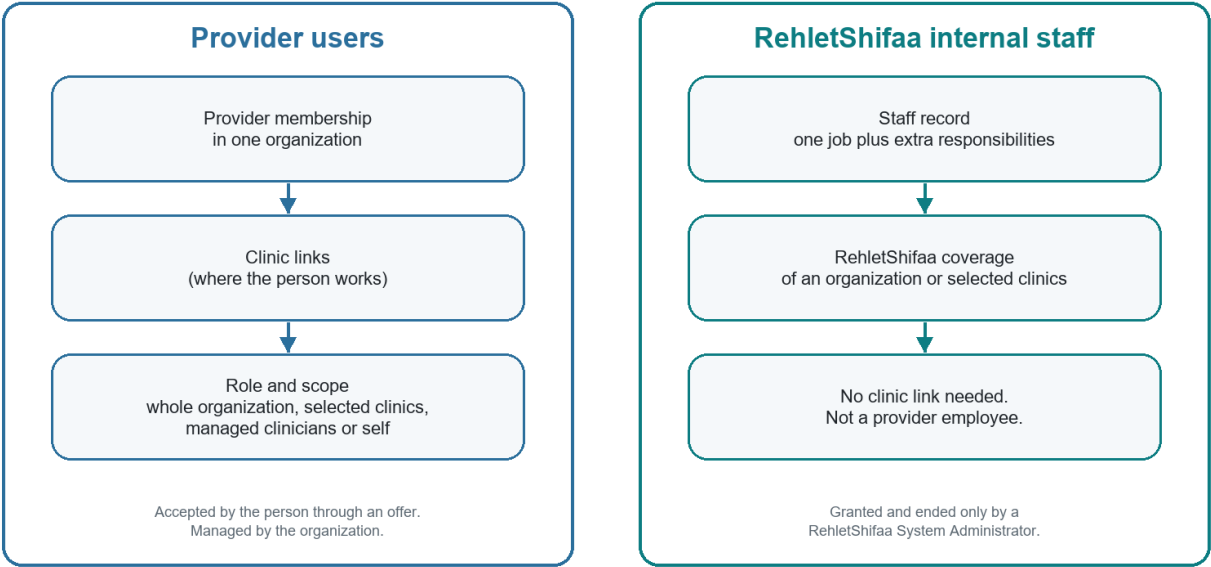
Figure 1 One person can work across organizations and clinics

- The sign-in identity and the practitioner profile are shared across the platform.
- Each organization membership starts and ends on its own.
- Each clinic link and each role names exactly where it applies.

Two ways to reach a provider organization

Provider staff reach their organization through a provider membership. RehletShifaa employees who serve that organization, for example a Credentialing Specialist, reach it through RehletShifaa coverage. The two never mix.

Two separate ways to reach a provider organization



Providers never see, change or remove RehletShifaa coverage

Figure 2 Provider membership and RehletShifaa coverage are separate

	Provider membership	RehletShifaa coverage
Who has it	Organization Owners, Practice Managers, Consultants, Associate Doctors, Consultant Assistants	RehletShifaa staff such as Provider Operations, Credentialing Specialists and Care Coordination
How it starts	The person accepts an offer	A System Administrator grants it
Needs a clinic link	Yes, for clinic-specific roles	No
Visible to the provider	Yes, in the organization's people lists	No. Shown only in RehletShifaa's Control Center
Who can end it	The organization's authorized managers	Only a RehletShifaa System Administrator
Can one person have both?	No. A RehletShifaa employee cannot also be a provider member (adopted default).	No, for the same reason.

4 Platform governance roles

These roles govern the RehletShifaa platform itself. They are separate from daily care work and from owning a provider organization.

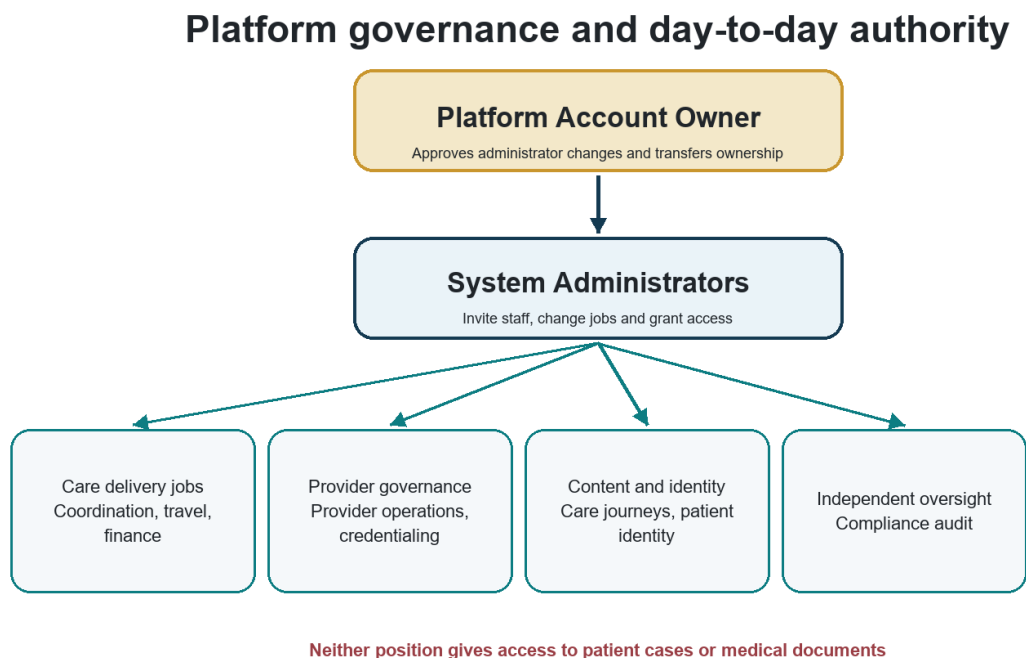


Figure 3 Governance and daily work are separate

Platform Account Owner

The contractual root of trust for the RehletShifaa platform. Ownership is a protected relationship, not a role that anyone can grant.

Role fact	Plain-language meaning
Where it applies	Platform governance actions only.
May do	Approve or reject System Administrator appointments and removals; transfer ownership; authorize the first administrators at handover; restore administration when no administrator is left; remove an administrator in an emergency; cancel a vendor recovery; read the governance summary.
Must not do	Read patient cases or documents; run finance or provider operations; grant ordinary staff jobs; act as the owner of any provider organization because of platform ownership.
Who grants it	Not granted like a role. The first owner is named at installation and claims ownership with strong sign-in. After that, only the current owner can nominate a successor, and the successor must accept.
Who removes it	Ends only when a nominated successor accepts, or when a vendor recovery completes. A System Administrator can never transfer or remove ownership.
Strong sign-in	Required for every owner action. Sensitive actions also need a sign-in within the last five minutes.
Typical handoff	Approves administrator changes. System Administrators do the day-to-day staff and access work.
Tester focus	Ownership alone gives no clinical, operational, financial, provider or patient access. Transfer and emergency actions are recorded.

Transferring platform ownership

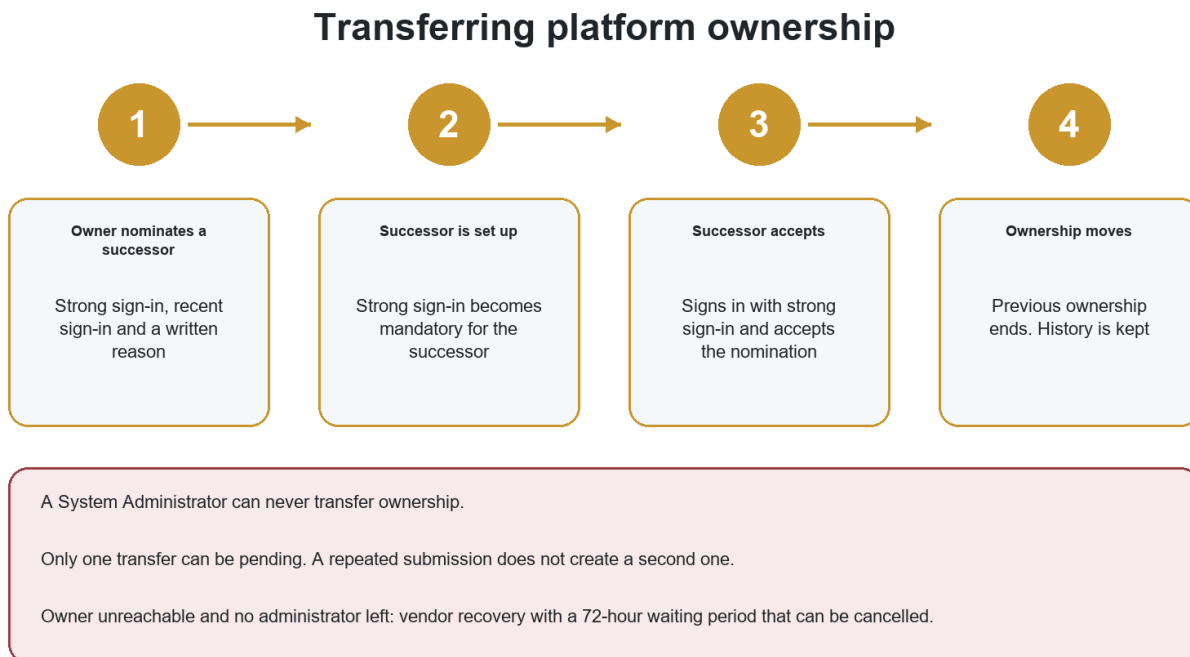


Figure 4 Ownership moves only when the successor accepts

Vendor recovery (adopted default: 72-hour waiting period) is for the rare case where the owner cannot be reached and no administrator remains. It is verified outside the platform, everyone is notified, and it can be cancelled during the waiting period.

System Administrator

Manages RehletShifaa staff and access without becoming a clinical superuser. Two active holders are recommended.

Role fact	Plain-language meaning
Where it applies	Platform access governance and staff administration.
May do	Invite staff; change a person's job; add or remove compatible extra responsibilities; grant and end RehletShifaa coverage of provider organizations; view effective access and the access audit; request or approve another administrator's appointment or removal.
Must not do	Approve their own request; grant or remove their own access; open patient cases or documents; make finance, credential, patient identity, provider or care journey decisions; bypass role conflicts; remove the last administrator; give someone a job through the People page instead of a job change.
Who grants it	An administrator requests the appointment and another administrator or the owner approves it (see the table below). The person must already be an active RehletShifaa staff member.
Who removes it	A removal request approved by another administrator or the owner; the person's own resignation; or the owner's emergency removal. The last administrator cannot be removed except by the owner's emergency removal.
Strong sign-in	Required on every request. Administrator changes, access grants and staff invitations also need a sign-in within the last five minutes.

Role fact	Plain-language meaning
Typical handoff	Hands operational work to the right staff job. Another administrator or the owner approves administrator changes.
Tester focus	No self-approval, no clinical access, last-administrator protection, and owner recovery when no administrator remains.

Administrator appointment and removal rules

Situation	Who starts it	Who approves it	Protection
Handover at installation	The vendor's installation file names the first administrators	The owner, when claiming ownership	The vendor keeps no account after handover
Two or more administrators	Any administrator	Another administrator (not the person affected) or the owner	Requester, approver and person affected must differ
Exactly one administrator	That administrator	The owner	The last administrator stays until the replacement is active
No administrator after handover	The owner ("Restore administration")	No second approval	Strong sign-in, recent sign-in, reason and a critical notice
Owner unreachable and no administrator	Vendor recovery	Waiting period, can be cancelled	Verified outside the platform
Normal removal	Any administrator	Another administrator or the owner	Cannot remove the last administrator
Resignation	The administrator	None	Cannot remove the last administrator
Emergency removal	The owner	None	Explicit warning; may leave zero administrators until restored
Owner also becomes an administrator	Another administrator	Another administrator	Never self-requested or self-approved (adopted default: allowed)

A request that nobody approves expires after 72 hours (adopted default). The screens say: "Add the replacement first, then remove."

5 RehletShifaa internal staff roles

Internal staff are RehletShifaa employees. They stay RehletShifaa employees even when they serve a provider organization, never appear in a provider's people lists, and cannot be invited into a provider organization.

How internal jobs work

- Each staff member has exactly one job. The only exception is a person who is a System Administrator and nothing else ("administration only").
- Operational jobs (coordination, travel, finance and provider operations) can only be a person's main job, because they carry work queues and reporting lines.
- Five independent functions can be a job or an extra responsibility: Credentialing Specialist, Patient Identity Reviewer, Care Journey Manager, Care Journey Approver and Compliance Auditor. System Administrator can also be added as a protected extra responsibility.
- Some combinations are always refused for one person: Credentialing Specialist with Provider Operations; Care Journey Manager with Care Journey Approver; Compliance Auditor with System Administrator.

- The job title on screen is for display. Access comes from the person's assignments and their scope.
- A new staff member has no access until their first sign-in with strong sign-in.

Care Coordination Manager

Leads the coordination team and supervises patient cases for the provider organizations the team serves.

Role fact	Plain-language meaning
Where it applies	The whole platform for coordination work, limited to the provider organizations the manager serves.
May do	Supervise coordination work; view team queues; transfer cases; set up coordination pools and routing rules for the organizations served; do coordination casework.
Must not do	Act for organizations not served; do finance, travel, credentialing, provider administration or access administration.
Who grants it	A System Administrator, as this person's job, in the staff invitation or through a job change. It cannot be added as an extra responsibility. The organizations and clinics served are RehletShifaa coverage, also granted and ended only by a System Administrator.
Who removes it	A System Administrator through a job change or by removing the responsibility, or through offboarding, which the person's manager or a System Administrator starts. A job change is refused while the person still owns work that the new job cannot handle.
Strong sign-in	Required on every request. Before first use the person enrolls an authenticator.
Typical handoff	Receives staff setup from a System Administrator; hands cases to Care Coordinators and other teams.
Tester focus	Team visibility, transfer and organization boundaries. Promoting a Care Coordinator to this job is not blocked by the cases they own.

Care Coordinator

Owns and moves forward assigned coordination cases, connecting patients, providers and internal teams.

Role fact	Plain-language meaning
Where it applies	Coordination work for the provider organizations served. Which cases they can open depends on case ownership and routing.
May do	Take ownership of eligible cases; work assigned coordination tasks; hand work to the next team as the workflow requires.
Must not do	Supervise the whole team; configure pools; open unrelated cases; do travel, finance, credentialing or administration work.
Who grants it	A System Administrator, as this person's job, in the staff invitation or through a job change. It cannot be added as an extra responsibility. The organizations and clinics served are RehletShifaa coverage, also granted and ended only by a System Administrator.
Who removes it	A System Administrator through a job change or by removing the responsibility, or through offboarding, which the person's manager or a System Administrator starts. A job change is refused while the person still owns work that the new job cannot handle.
Strong sign-in	Required on every request. Before first use the person enrolls an authenticator.
Typical handoff	Escalates or transfers to the Care Coordination Manager; hands approved work to travel, finance or provider teams.

Role fact	Plain-language meaning
Tester focus	Access to owned and assigned cases; denial for unrelated cases and organizations.

Travel and Logistics Manager

Supervises travel and logistics work for patient journeys.

Role fact	Plain-language meaning
Where it applies	Travel casework across the platform, with supervision.
May do	Supervise travel cases; review team work; see the team's travel work; manage handoffs within travel.
Must not do	Change clinical decisions; set finance policy; manage provider membership; open case data beyond what travel work needs.
Who grants it	A System Administrator, as this person's job, in the staff invitation or through a job change. It cannot be added as an extra responsibility.
Who removes it	A System Administrator through a job change or by removing the responsibility, or through offboarding, which the person's manager or a System Administrator starts. A job change is refused while the person still owns work that the new job cannot handle.
Strong sign-in	Required on every request. Before first use the person enrolls an authenticator.
Typical handoff	Receives travel needs from coordination and hands completed arrangements back.
Tester focus	Supervisory view limited to travel work.

Travel and Logistics Specialist

Plans and records travel arrangements for assigned cases.

Role fact	Plain-language meaning
Where it applies	Assigned travel cases.
May do	Build travel plans; record arrival and logistics milestones; complete assigned operations steps.
Must not do	Supervise the travel team; open unrelated cases; approve finance or clinical decisions.
Who grants it	A System Administrator, as this person's job, in the staff invitation or through a job change. It cannot be added as an extra responsibility.
Who removes it	A System Administrator through a job change or by removing the responsibility, or through offboarding, which the person's manager or a System Administrator starts. A job change is refused while the person still owns work that the new job cannot handle.
Strong sign-in	Required on every request. Before first use the person enrolls an authenticator.
Typical handoff	Works from coordination requirements and passes completed arrangements on.
Tester focus	Access limited to assigned travel cases.

Commercial and Finance Manager

Owns commercial policy and supervises the financial side of patient cases.

Role fact	Plain-language meaning
Where it applies	Finance casework and commercial policy across the platform.
May do	Set margin and deposit policy; manage exchange rates and service templates; supervise finance casework; do finance work when needed.
Must not do	Open unrelated clinical material; grant access; make credential decisions.
Who grants it	A System Administrator, as this person's job, in the staff invitation or through a job change. It cannot be added as an extra responsibility.
Who removes it	A System Administrator through a job change or by removing the responsibility, or through offboarding, which the person's manager or a System Administrator starts. A job change is refused while the person still owns work that the new job cannot handle.
Strong sign-in	Required on every request. Before first use the person enrolls an authenticator.
Typical handoff	Sets the policy that Finance Officers apply and handles exceptions.
Tester focus	Manager-only policy, exchange-rate and template actions.

Finance Officer

Carries out the financial steps of assigned cases under the approved commercial policy.

Role fact	Plain-language meaning
Where it applies	Assigned finance cases.
May do	Record deposits and payments; approve commercial terms for assigned cases; complete assigned finance tasks.
Must not do	Change margin, deposit, exchange-rate or template policy; open unrelated cases; administer users.
Who grants it	A System Administrator, as this person's job, in the staff invitation or through a job change. It cannot be added as an extra responsibility.
Who removes it	A System Administrator through a job change or by removing the responsibility, or through offboarding, which the person's manager or a System Administrator starts. A job change is refused while the person still owns work that the new job cannot handle.
Strong sign-in	Required on every request. Before first use the person enrolls an authenticator.
Typical handoff	Receives approved terms and hands financial completion back to the patient journey.
Tester focus	Assigned-case limits; denial for policy changes.

Provider Operations Manager

Leads the setup and operational governance of provider organizations.

Role fact	Plain-language meaning
Where it applies	Creating providers across the platform, plus the organizations or clinics this person covers.

Role fact	Plain-language meaning
May do	Create provider organizations with their first clinic; activate or suspend providers; manage provider and clinic setup; send membership offers for covered organizations; manage direct consultant setup.
Must not do	Decide credentials for a clinician they invited or manage; become a provider employee; open patient cases.
Who grants it	A System Administrator, as this person's job, in the staff invitation or through a job change. It cannot be added as an extra responsibility. The organizations and clinics served are RehletShifaa coverage, also granted and ended only by a System Administrator.
Who removes it	A System Administrator through a job change or by removing the responsibility, or through offboarding, which the person's manager or a System Administrator starts. A job change is refused while the person still owns work that the new job cannot handle.
Strong sign-in	Required on every request. Before first use the person enrolls an authenticator.
Typical handoff	Prepares providers, then hands credential decisions to an independent Credentialing Specialist.
Tester focus	Coverage boundaries; independence from credentialing.

Provider Operations Specialist

Sets up providers and clinicians for covered organizations under the manager's oversight.

Role fact	Plain-language meaning
Where it applies	Covered organizations or clinics.
May do	View and update covered providers; send membership offers; perform approved direct setup tasks.
Must not do	Activate or suspend providers; decide credentials for clinicians they invited or manage; act outside coverage.
Who grants it	A System Administrator, as this person's job, in the staff invitation or through a job change. It cannot be added as an extra responsibility. The organizations and clinics served are RehletShifaa coverage, also granted and ended only by a System Administrator.
Who removes it	A System Administrator through a job change or by removing the responsibility, or through offboarding, which the person's manager or a System Administrator starts. A job change is refused while the person still owns work that the new job cannot handle.
Strong sign-in	Required on every request. Before first use the person enrolls an authenticator.
Typical handoff	Prepares the organization and clinicians for independent credential review and manager activation.
Tester focus	No activation authority; no credential decisions for their own invitees.

Credentialing Specialist

Makes the independent credential decision for clinicians, separately for each provider organization.

Role fact	Plain-language meaning
Where it applies	Covered organizations or clinics.

Role fact	Plain-language meaning
May do	View credential files; review evidence; ask for more information; verify, reject or suspend credentials, including the level (Consultant or Associate Doctor) for that organization; decide direct clinician credentials.
Must not do	Review their own credentials; decide on a clinician they invited or manage; hold a Provider Operations job at the same time.
Who grants it	A System Administrator, either as the person's job or as an additional responsibility for someone who already has another compatible job. The organizations and clinics served are RehletShifaa coverage, also granted and ended only by a System Administrator.
Who removes it	A System Administrator through a job change or by removing the responsibility, or through offboarding, which the person's manager or a System Administrator starts. A job change is refused while the person still owns work that the new job cannot handle.
Strong sign-in	Required on every request. Before first use the person enrolls an authenticator.
Typical handoff	Receives a complete file from provider operations or the clinician and returns an independent decision.
Tester focus	Self-review denial, inviter and reviewer separation, and clinic-level coverage.

Patient Identity Reviewer

Reviews patients' legal identity evidence and records a decision.

Role fact	Plain-language meaning
Where it applies	The patient identity review queue.
May do	Open the identity review queue; inspect the identity evidence; approve, reject or ask for more information.
Must not do	Read clinical documents or unrelated case details; administer staff; make credential decisions.
Who grants it	A System Administrator, either as the person's job or as an additional responsibility for someone who already has another compatible job.
Who removes it	A System Administrator through a job change or by removing the responsibility, or through offboarding, which the person's manager or a System Administrator starts. A job change is refused while the person still owns work that the new job cannot handle.
Strong sign-in	Required on every request. Before first use the person enrolls an authenticator.
Typical handoff	Returns the identity decision to the patient journey.
Tester focus	Sees identity evidence only, never general clinical documents.

Care Journey Manager

Designs and prepares reusable care journey templates.

Role fact	Plain-language meaning
Where it applies	Care journey design.
May do	Create and edit draft journeys; validate and simulate them; submit them for approval.

Role fact	Plain-language meaning
Must not do	Approve, publish or retire their own journey; also hold Care Journey Approver.
Who grants it	A System Administrator, either as the person's job or as an additional responsibility for someone who already has another compatible job.
Who removes it	A System Administrator through a job change or by removing the responsibility, or through offboarding, which the person's manager or a System Administrator starts. A job change is refused while the person still owns work that the new job cannot handle.
Strong sign-in	Required on every request. Before first use the person enrolls an authenticator.
Typical handoff	Submits a finished draft to a Care Journey Approver.
Tester focus	Author and approver separation; no patient case access.

Care Journey Approver

Independently approves and publishes care journey templates.

Role fact	Plain-language meaning
Where it applies	Care journey approval.
May do	Review submitted journeys; approve, publish or retire journey versions.
Must not do	Author the journey they approve; also hold Care Journey Manager; open patient cases.
Who grants it	A System Administrator, either as the person's job or as an additional responsibility for someone who already has another compatible job.
Who removes it	A System Administrator through a job change or by removing the responsibility, or through offboarding, which the person's manager or a System Administrator starts. A job change is refused while the person still owns work that the new job cannot handle.
Strong sign-in	Required on every request. Before first use the person enrolls an authenticator.
Typical handoff	Receives a submitted journey and publishes or rejects it.
Tester focus	Maker and checker separation; no patient case access.

Compliance Auditor

Independently reviews roles, assignments and who can do what.

Role fact	Plain-language meaning
Where it applies	Access audit across the platform.
May do	View role definitions; view effective access; review the access audit history.
Must not do	Open patient cases or documents; administer staff; change roles; also be a System Administrator. Case-level clinical audit is a future capability, not part of this release.
Who grants it	A System Administrator, either as the person's job or as an additional responsibility for someone who already has another compatible job.

Role fact	Plain-language meaning
Who removes it	A System Administrator through a job change or by removing the responsibility, or through offboarding, which the person's manager or a System Administrator starts. A job change is refused while the person still owns work that the new job cannot handle.
Strong sign-in	Required on every request. Before first use the person enrolls an authenticator.
Typical handoff	Reviews evidence produced by access governance without changing it.
Tester focus	Read-only audit access; no clinical access; conflict with System Administrator.

6 Provider organizations, clinics and roles

A provider organization is the legal or contracting party. A facility is one operating site inside that organization; the screens call facilities "Clinics & branches". Each clinic belongs to exactly one organization.

One organization with several clinics



Figure 5 Organization-wide roles reach every clinic; clinic-specific roles reach only the named clinics

- An organization can have any number of clinics, hospitals, medical centers, diagnostic centers, rehabilitation centers and virtual-care sites.
- A site that is a separate legal or contracting party is its own provider organization, not a branch.
- A clinic is Onboarding, Active, Suspended or Closed. A physical clinic cannot become active without an address.
- An organization is created together with its first clinic, which becomes its default clinic.
- An organization cannot be active without at least one active clinic.
- Exactly one clinic is the default. It pre-fills forms and anchors older records. It never grants access by itself.
- Closing the default clinic requires choosing a replacement; otherwise the organization shows "default clinic required". Suspending the default clinic does not move the default.
- Suspending one clinic stops work at that clinic only. Sibling clinics carry on.

Existing providers after the upgrade

Each existing organization receives one default clinic, created in the Onboarding state because today's records hold no address.

The organization owner or RehletShifaa Provider Operations must add the address (and confirm the clinic type where flagged) before that clinic can become active.

Existing staff and clinicians are linked to the default clinic. Organization-wide roles keep working unchanged.

Organization-wide versus clinic-specific

Action	Organization-wide manager	Manager of Dubai and Sharjah only
Work at Dubai or Sharjah	Yes	Yes
Work at Abu Dhabi Hospital	Yes	No
Work at a clinic opened next month	Yes, automatically	No, until the clinic is added to the role
Change organization defaults (for example default prices)	Yes, if the role allows it	No. Organization-level settings need an organization-wide role
Manage clinicians	Clinicians anywhere in the organization	Only managed clinicians at Dubai and Sharjah
Invite new members	Only if member invitation has been granted	Only if member invitation has been granted

Organization Owner

Owns the provider organization and governs its members and operations. This is a different role from the Platform Account Owner.

Role fact	Plain-language meaning
Where it applies	The whole organization, including every current and future clinic.
May do	Manage provider members and organization-level settings; send membership offers; manage clinics and branches; hold other compatible provider roles where the rules allow.
Must not do	Grant platform or RehletShifaa staff access; see, change or remove RehletShifaa coverage; bypass self-approval rules; give themselves a role; see another organization; do clinical work without the clinical qualification and credential checks.
Who grants it	Given through a membership offer that includes this role and that the person accepts. Offers can come from RehletShifaa Provider Operations for an organization it covers, or from an authorized manager of the organization. Who may grant this role to an existing member is not yet decided (Section 14).
Who removes it	Not yet decided: who may remove an Organization Owner, whether an organization must always keep at least one, and how ownership passes to someone else (Section 14). Ending the person's membership ends the role.
Strong sign-in	Required at launch, on every request.
Typical handoff	Delegates daily work to Practice Managers and clinical work to eligible clinicians.

Role fact	Plain-language meaning
Tester focus	Reach to every clinic including new ones; denial of every platform, staff and coverage action; no self-granting.

Practice Manager

Runs provider operations for one, several or all clinics of the organization.

Role fact	Plain-language meaning
Where it applies	Organization-wide, or only the listed clinics. There is no separate Branch Manager role.
May do	Manage operations and managed clinicians within scope; manage schedules, services and prices where the role allows; invite members only if member invitation has been granted separately.
Must not do	Act at an unlisted sibling clinic; change organization-level settings when clinic-limited; grant platform access; approve a price they wrote; bypass qualification or clinic-link rules.
Who grants it	Offer acceptance, or a role grant by an authorized provider manager (normally the Organization Owner). Nobody can grant a role to themselves.
Who removes it	The same authorized provider managers. A clinic-limited role loses a clinic when the person's link to that clinic ends, and ends if no clinic remains.
Strong sign-in	Required at launch, on every request.
Typical handoff	Coordinates staff, clinicians and clinic operations within scope.
Tester focus	Test both variants. A clinic-limited manager is denied at sibling clinics and on organization defaults.

Consultant

Provides consultant-level services in the organization and clinics named in the role.

Role fact	Plain-language meaning
Where it applies	Listed clinics where the person has an active clinic link, or organization-wide when granted that way; plus their own professional workspace.
May do	Work as a consultant at those clinics; manage their own schedule and eligible services; supervise Associate Doctors through an active supervision relationship.
Must not do	Work as a consultant without a Consultant declaration and a Consultant-level credential decision for this organization; work at a clinic without an active clinic link; approve their own credentials or a price they wrote.
Who grants it	Offer acceptance or a grant by an authorized provider manager, and only when the checks in Section 8 pass.
Who removes it	The same provider managers; ending the clinic link or membership; or the credential being suspended.
Strong sign-in	Required at launch (confirmed business decision).
Typical handoff	Credential decisions come from an independent RehletShifaa Credentialing Specialist.
Tester focus	Declared level, organization credential level, active clinic link and role scope must all be present.

Associate Doctor

Provides clinical services under the supervision of an eligible Consultant.

Role fact	Plain-language meaning
Where it applies	Listed clinics with an active clinic link, plus their own workspace and the supervision relationship.
May do	Work as an Associate Doctor at those clinics under an active supervisor; maintain their own workspace.
Must not do	Work without an active supervising Consultant in the same organization; act as a Consultant there; act at another clinic or organization without the matching role and clinic link.
Who grants it	Offer acceptance or a grant by an authorized provider manager, only with an Associate or Consultant credential decision for this organization and an active supervisor.
Who removes it	The same provider managers; ending the clinic link or membership. If supervision ends, the role cannot be used until a supervisor is in place again.
Strong sign-in	Required at launch (confirmed business decision).
Typical handoff	Works under the supervising Consultant and the organization's credential decision.
Tester focus	Active supervision is mandatory; a Consultant-level credential also satisfies the Associate requirement.

Consultant Assistant

Supports selected clinicians and clinic operations without becoming a clinician.

Role fact	Plain-language meaning
Where it applies	Listed clinics, and the clinicians the assistant is explicitly assigned to support.
May do	Support the assigned clinicians and clinics; perform allowed administrative tasks within that scope.
Must not do	Hold Consultant or Associate Doctor at the same clinic; do clinical work through this role; act for clinicians or clinics not assigned.
Who grants it	Offer acceptance or a grant by an authorized provider manager.
Who removes it	The same provider managers; ending the clinic link or membership.
Strong sign-in	Not required by this role itself. Required if the same person holds any role that needs it, anywhere on the platform.
Typical handoff	Escalates clinical decisions to the responsible clinician.
Tester focus	Same-clinic conflict with clinical roles; denial outside assigned clinicians and clinics.

Clinical work in the first release

Provider clinicians see the case summary made available for cases assigned to them in their organization and clinic. Wider clinical capabilities (clinical records, appointments) are future work.

Wherever this guide says a Consultant may "work" or "consult", it means the provider capabilities that exist in that release, always within these rules.

7 Patients and patient representatives

The patient experience is not changed by this model. These roles are listed so that readers can see how they relate to the others.

Patient

Uses the patient portal for their own identity, care requests, documents, messages and journey steps.

Role fact	Plain-language meaning
Where it applies	Their own care.
May do	Use the patient portal; keep permitted profile details up to date; see and act on their own care; keep patient access after joining a provider organization.
Must not do	Receive provider, staff or platform authority from the Patient role.
Who grants it	Created through patient sign-up and account activation.
Who removes it	Unchanged by this model.
Strong sign-in	Password sign-in is enough for patient use. If the same person later takes on a role that needs strong sign-in (for example Organization Owner), strong sign-in then applies to all their sign-ins.
Typical handoff	Works with care coordination, providers, travel and finance through the patient journey.
Tester focus	Joining a provider organization does not remove Patient access; provider authority does not start before acceptance.

Patient Representative

Acts for a patient in the patient portal, for example a relative who submits the case.

Role fact	Plain-language meaning
Where it applies	The represented patient's cases, where the representative's authorization is recorded.
May do	Use the portal for the represented patient's permitted actions.
Must not do	Act for unrelated patients; gain provider, staff or platform authority from this role.
Who grants it	Existing patient-portal representative flow. Relationship-based representative grants are future work.
Who removes it	Unchanged by this model.
Strong sign-in	Same as Patient.
Typical handoff	Communicates with the care team within the recorded authorization.
Tester focus	Existing behaviour only (regression test): no access to other patients' information.

Things that are not business roles

Item	What it is	Why it is not a role
"Strong sign-in required" marker	A sign-in setting that forces a second factor	It never grants any business permission

Item	What it is	Why it is not a role
Vendor operator	The installer used during setup or recovery	Not a platform user; it leaves after handover
"Provider user"	A group label	Access comes from Organization Owner, Practice Manager, Consultant, Associate Doctor or Consultant Assistant roles
"Internal staff"	A group label	Access comes from the person's staff job, extra responsibilities and coverage

Not in the launch catalogue

- Support Specialist and Support Manager: deferred.
- Credentialing Manager: not a separate role.
- Journey Specialist roles: not included.
- Platform Administrator, Access Governance Manager and Support Agent: retired.
- Branch Manager: not a role. A Practice Manager limited to certain clinics covers it.
- The old sign-in system staff roles: removed before go-live; they never grant access in production.

8 Scopes, role combinations and professional eligibility

Where a role applies

A role without the right scope cannot reach the record. The platform works out the organization and clinic from the stored record, never from what the browser sends.

Scope	Plain meaning	Used by
Organization	The whole provider organization, including clinics added later	Organization Owner; organization-wide Practice Manager; clinicians granted organization-wide
Selected clinics (provider)	Only the listed clinics of one organization, where the person has an active clinic link	Clinic-limited Practice Manager, Consultant, Associate Doctor, Consultant Assistant
Managed clinicians	Only clinicians linked through an active management relationship	Practice Managers and support roles
Self	Only the person's own profile and professional records	Every clinician and assistant
Platform	A named capability across RehletShifaa	System Administrator and platform-wide staff jobs
Covered organizations	Only the provider organizations assigned to a RehletShifaa staff member	Provider Operations, Credentialing, Care Coordination
Covered clinics	Only listed clinics of a covered organization. No clinic link needed	Provider Operations and Credentialing when limited to certain clinics

Which individual cases a coordinator, travel or finance staff member can open is decided by case ownership and assignment in the workflow, not by a separate scope.

Internal staff combinations

Combination	Result
One job	Required. Only a person who is a System Administrator and nothing else may have no job.
Extra responsibilities	Credentialing Specialist, Patient Identity Reviewer, Care Journey Manager, Care Journey Approver, Compliance Auditor and System Administrator, when compatible (adopted default).
Credentialing Specialist with Provider Operations	Refused. Whoever invites or manages clinicians cannot also decide their credentials.
Care Journey Manager with Care Journey Approver	Refused. The author cannot approve their own work.
Compliance Auditor with System Administrator	Refused. The auditor cannot audit their own administration.
Any internal job with a provider role	Refused. RehletShifaa staff are never provider members (adopted default).

Provider role combinations

Held by one person	Result
Organization Owner and Practice Manager	Allowed.
Organization Owner or Practice Manager with Consultant Assistant	Allowed.
Organization Owner or Practice Manager with Consultant	Allowed when the clinical checks pass. No self-credentialing and no approving prices they wrote.
Organization Owner or Practice Manager with Associate Doctor	Allowed when the clinical checks and supervision are in place.
Consultant and Associate Doctor	Allowed only at different clinics or in different organizations, for a Consultant-qualified person with an active supervisor for the Associate work (adopted default).
Consultant Assistant with Consultant or Associate Doctor	Refused at the same clinic.

Four facts needed for clinical work

1. Declared level. The professional states, once for the whole platform, whether they are a Consultant or an Associate Doctor. A level carried over from the old system must be confirmed by the professional before any new Consultant role.
2. Credential decision for this organization. An independent RehletShifaa Credentialing Specialist verifies the level separately for each organization. The organization cannot verify its own clinicians, and evidence is not shared between organizations automatically.
3. Local role. The organization grants a Consultant or Associate Doctor role for the organization or for listed clinics.
4. Clinic link. The person has an active link to every clinic named in the role.

No local role can raise the declared or verified level. A Consultant role needs a Consultant declaration and a Consultant-level credential decision in that organization. An Associate Doctor role accepts an Associate or

Consultant credential decision and needs an active supervising Consultant in the same organization. A person with no declared level cannot receive any clinical role.

9 One person in several organizations: signing in and switching

A person who works for several organizations signs in once with one account. In the provider workspace ("My Practice") they first choose an organization and then a clinic.

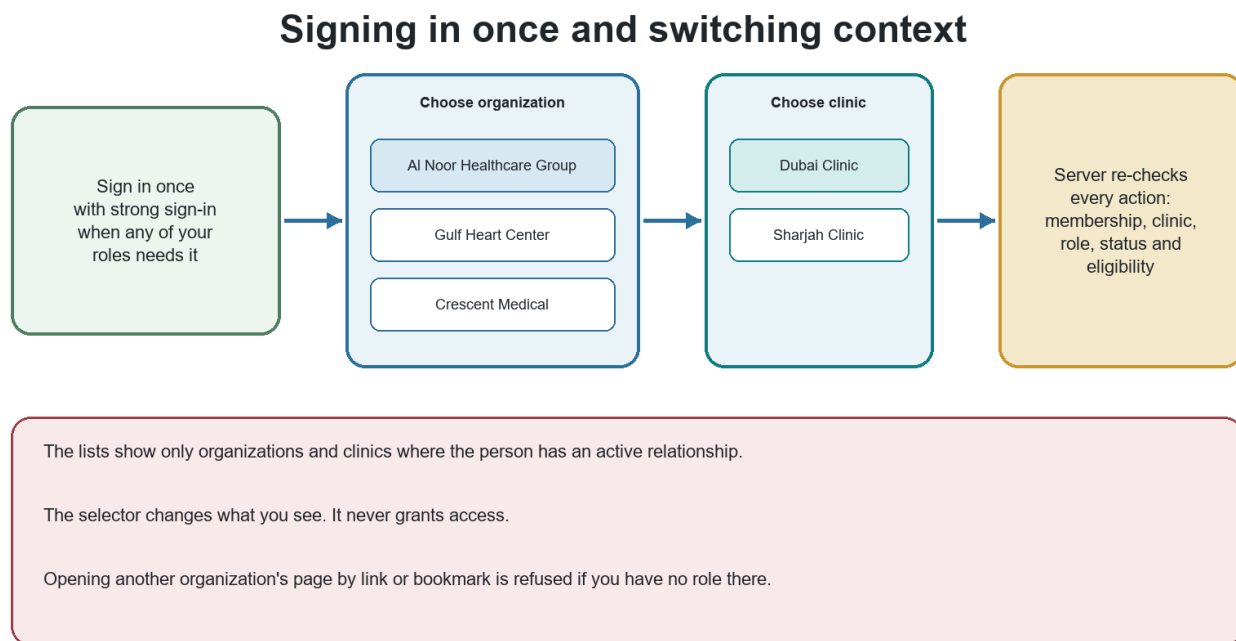


Figure 6 The selector changes the view; the server decides access

- The organization list shows only organizations where the person has an active membership. The clinic list shows only clinics they are linked to or reach through an organization-wide role.
- Every screen and every action is checked again on the server for that organization and clinic.
- A clinician page shows only the engagements the viewer is allowed to see. A Gulf Heart manager looking at Dr Sara sees her Gulf Heart work, not her Al Noor or Crescent work.
- Search, notifications and audit views are filtered the same way.
- Membership offers appear as a card with Accept and Decline.

Strong sign-in belongs to the person, not to one context

If any of a person's roles anywhere needs strong sign-in, every sign-in by that person needs it, including when they work in a context that would not need it on its own. The requirement is removed only when the last role that needs it ends.

Person	Strong sign-in?	Why
Dr Sara (Owner at Al Noor, Associate Doctor at Crescent)	Always	Owner and clinician roles both require it
A patient who accepts an Organization Owner offer	Always, from acceptance	The new role requires it; it also applies to patient use
A Consultant Assistant with no other role	No	The role does not require it

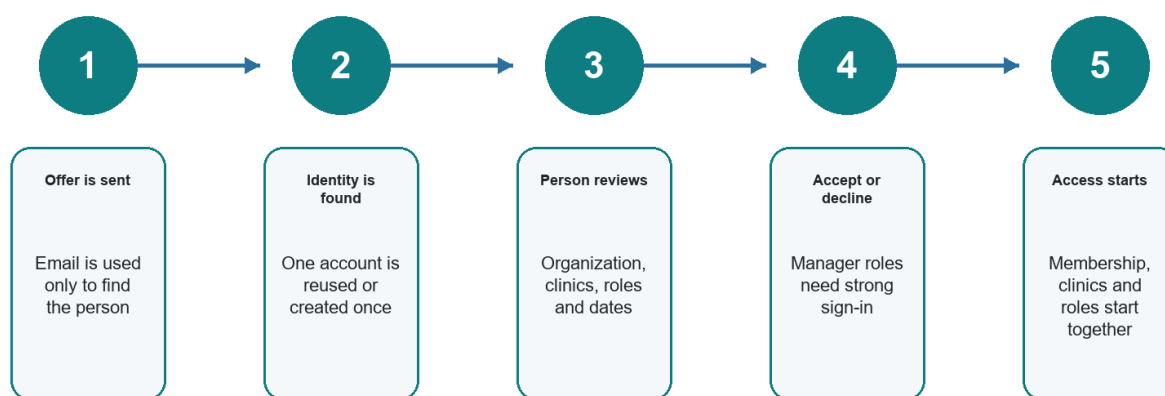
Person	Strong sign-in?	Why
A Practice Manager whose last manager role ends, and who is not a clinician	No longer	Removed when no qualifying role remains

10 Joining, activation, suspension and leaving

Joining a provider organization: offers

Typing an email never attaches a person to an organization. The organization sends an offer; the person sees the organization, clinics, proposed roles and dates, and then accepts or declines.

Joining a provider organization needs the person's consent



Nothing is active before acceptance

The inviter always sees "Invitation sent", whether or not the person already has an account

Declining or letting one offer expire changes nothing in other organizations

Figure 7 Provider access starts only after the person accepts

Offer state	Meaning
Awaiting identity	The platform is finding or creating the one account for this email.
Pending	The person can review and accept or decline. Nothing is active yet.
Accepted	Membership, clinic links and roles start together. A practitioner profile is created once, if needed.
Declined	This offer ends. Nothing else changes.
Expired	Not answered in time. Adopted default: 14 days.
Cancelled	The sender withdrew the offer before acceptance.
Review required	The email matches more than one account in a way that cannot be resolved safely, or belongs to a RehletShifaa employee. RehletShifaa governance reviews it; by default it is refused.

- The sender always sees "Invitation sent", whether the email is new, belongs to an existing account or to a RehletShifaa employee. No organization can use invitations to find out who has an account.

- If a proposed role does not fit the person (for example Consultant for someone declared as Associate Doctor), the offer is still sent without that role and the sender sees a neutral "some roles need review" message.
- A manager role can only be accepted with strong sign-in.

Internal staff activation

1. A System Administrator creates the invitation with one job, any compatible extra responsibilities, and the operational setup (manager, pools, organizations served).
2. The invitee verifies their email, sets a password, enrolls an authenticator and signs in.
3. The first request made with strong sign-in activates the staff record, exactly once.
4. Until then the person has no access at all. A password-only sign-in never activates a staff record.

Suspension and ending: what changes and what stays

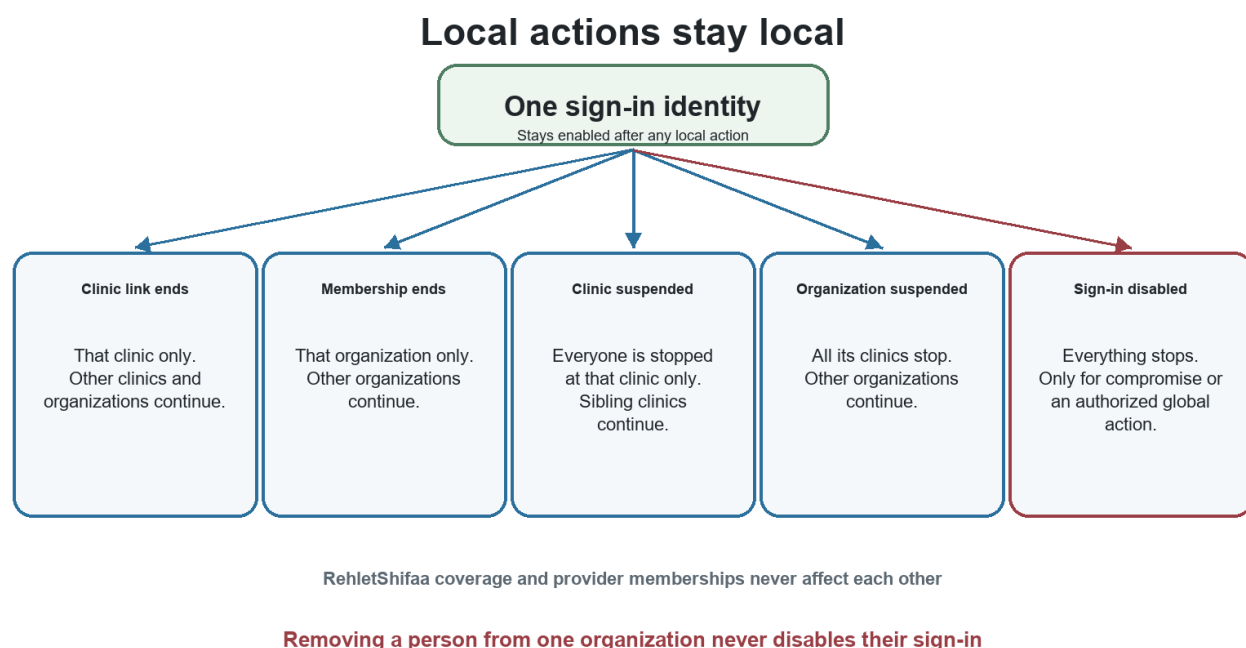


Figure 8 Local actions do not spread

Action	Who does it	What ends	What stays
End one clinic link	Authorized provider manager	That clinic from the person's clinic-limited roles; a role left with no clinic ends	Other clinics, organizations, the membership and RehletShifaa coverage
End one provider membership	Authorized provider manager	All roles, clinic links and relationships in that organization	Other organizations, the sign-in account, RehletShifaa coverage
End RehletShifaa coverage	System Administrator	That staff member's coverage of the organization or clinics	All provider memberships and clinic links
Suspend one clinic	Authorized provider manager or Provider Operations	Clinic-specific work at that clinic, for everyone	The organization and its other clinics
Suspend an organization	Provider Operations Manager	Work at all of its clinics, for everyone	Other organizations; history
Disable a person's sign-in	RehletShifaa governance only	Everything, everywhere	History, for audit

Not yet decided: a temporary pause of one person's membership in one organization (as opposed to ending it), and whether a provider member can end their own membership themselves. Today the model only defines ending a membership.

Internal staff leaving (offboarding)

- 1. The person's manager or a System Administrator starts offboarding with a reason. From then on the person receives no new work but keeps enough access to hand over.
- 2. If there is a security concern, sign-in is disabled immediately and handover continues without the person.
- 3. The platform lists blockers: owned cases, open tasks, travel or finance work, direct reports, and being the last System Administrator.
- 4. Authorized leads transfer the work using the normal Transfer and reassignment actions.
- 5. When nothing blocks, offboarding completes: access and coverage end, sign-in is disabled, sessions end, and all history is kept. The account is never deleted.

Changing an internal staff member's job

A job change replaces the old job with the new one in a single step, so the person never has both or neither. It is refused while the person owns work that the new job cannot handle. For example, a Care Coordinator becoming a Finance Officer must first transfer owned cases, while a Care Coordinator becoming a Care Coordination Manager keeps them.

11 Services, prices, schedules and cases

Services and prices

The organization owns the default service list. A clinic must explicitly offer a service before it can be priced, scheduled or booked there. The platform then uses the most specific active price.

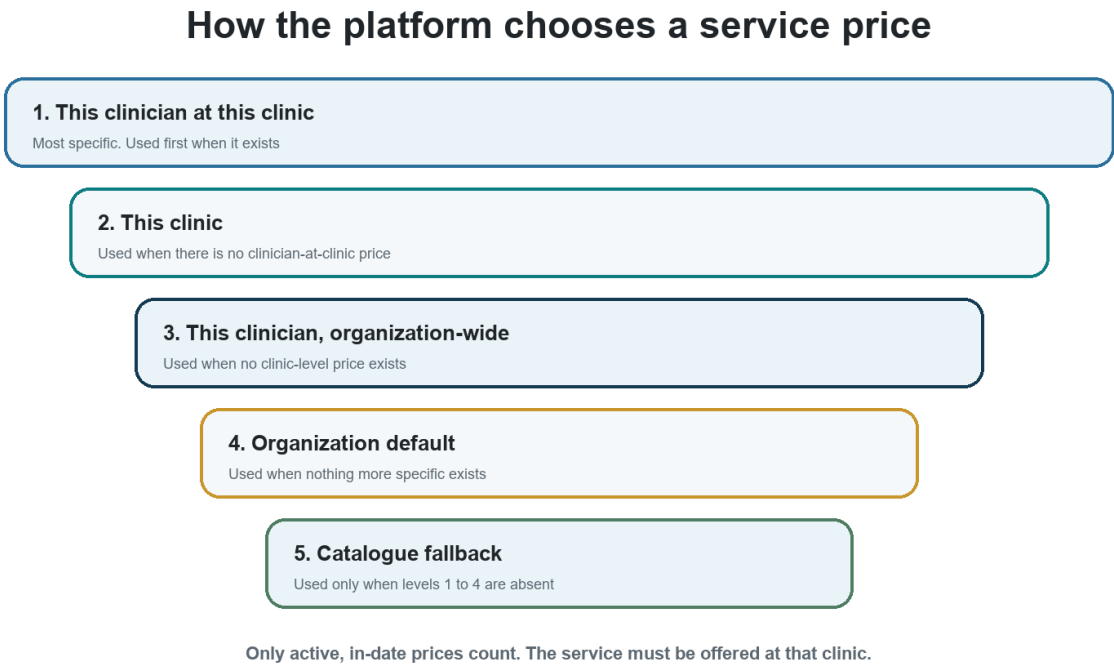


Figure 9 Price order, from most specific to fallback

Prices that exist for a consultation with Dr Sara	Price charged at Dubai	Price charged at Sharjah
Organization default 400; Sharjah clinic 450; Dr Sara organization-wide 500; Dr Sara at Dubai 550	550 (Dr Sara at Dubai)	450 (Sharjah clinic price beats Dr Sara's organization-wide price)

A clinician-specific price must be approved by someone other than its author.

Schedules and virtual care

- Every availability slot and exception names a clinic. Clinic closures override clinician slots at that clinic.
- Virtual care uses the organization's virtual-care clinic. An empty clinic never means virtual.
- A clinician cannot be booked in person at two clinics at the same time, or in person and virtually at the same time.
- Two virtual slots at the same time are refused unless that virtual clinic allows it (adopted default: not allowed).
- A clinician is never copied to represent another clinic, price or schedule.

Cases and routing

Moment in the case	What must be known
A provider Consultant is chosen	The provider organization is recorded on the case.
Intake and review	The clinic may still be open.
Before an in-person appointment, travel finalization or a location-specific service	The clinic (adopted default).
Care is virtual	Virtual is recorded explicitly, with the organization's virtual-care clinic.
A clinician is assigned	The clinician is active and eligible in the organization and linked to the case's clinic.
Routing picks a destination	Routing is by organization at launch; the clinic filters who is eligible (adopted default).

12 Tester handbook

Run these tests on a first-production-release build only

Every test here is an acceptance test for the first production release. On today's development system many will fail because the feature is not built (see Section 2).

A role test is incomplete if it checks only success. Every important permission needs a positive test in the right context and negative tests for the wrong organization, clinic, role, status, sign-in strength and qualification.

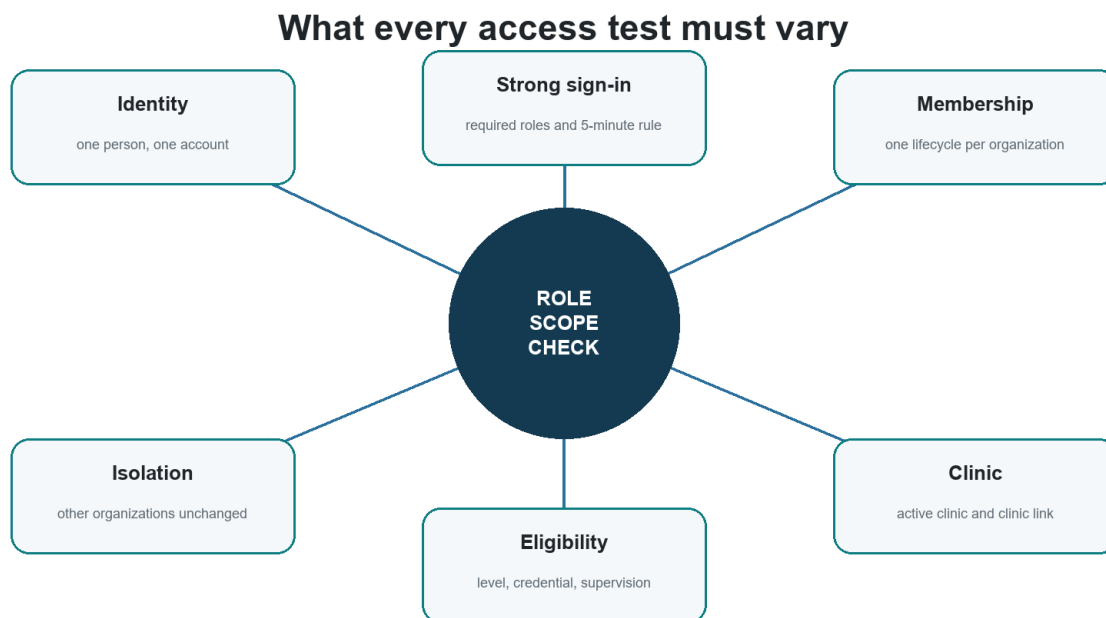


Figure 10 Vary each condition independently

Prerequisites

- A build that passes all first-production-release gates, with the sign-in system configured for strong sign-in.
- Authenticator secrets for QA accounts kept only in the local environment file, never in source control.
- Two separate browser profiles or private windows, so that two people can act at the same time.
- Read access to the access audit (a Compliance Auditor account) to collect evidence.

Test world

Organization	Clinics	Notes
Al Noor Healthcare Group	Dubai Clinic (default), Sharjah Clinic, Abu Dhabi Hospital, Al Noor Online (virtual)	Main organization for scope tests
Gulf Heart Center	Gulf Heart Main, Gulf Heart West	Second organization for isolation tests
Crescent Medical	Crescent Rehab	Third organization for supervision tests

Minimum test personas

Persona	Setup	Main purpose
Platform Account Owner	Active ownership; strong sign-in	Approvals; no business access
System Administrators A and B	Two active administrators	Maker and checker; last-administrator rules
Care Coordinator	Covers Al Noor; owns one case	Case access; job change blocking
Provider Operations Specialist	Covers Al Noor only	Setup limits; isolation from Gulf Heart
Credentialing Specialist (Omar)	Covers Al Noor Dubai Clinic only	RehletShifaa coverage tests

Persona	Setup	Main purpose
Organization Owner (Al Noor)	Organization-wide	Reach to all clinics; no platform access
Organization Owner (Gulf Heart)	Organization-wide	Cross-organization isolation
Practice Manager, organization-wide	Al Noor, organization scope	New-clinic inheritance
Practice Manager, clinic-limited	Al Noor, Dubai only	Sibling and organization-default denial
Dr Sara Haddad	As in the worked example (Section 13)	Multi-organization, multi-role
Supervising Consultant (Dr Lina Mansour)	Crescent, Consultant at Crescent Rehab	Supervision rules
Consultant Assistant	Al Noor, Dubai, assigned to one clinician	Assistant limits and conflicts
Patient and Patient Representative	Own case; one represented patient	Regression of patient access
Fresh email, internal staff email	No account; a RehletShifaa employee's email	Offer and identity tests

Identity and offers

Test	Action	Expected result
ID 01	Two organizations invite the same new email at the same time	One account and, after acceptance, one practitioner profile. Two separate pending offers.
ID 02	Invite a person who already has an account	A pending offer only. No membership, role or clinic link before acceptance.
ID 03	A provider invites a RehletShifaa employee's email	No offer becomes usable; the case goes to governance review and is refused by default.
ID 04	Invite a patient-only person	Normal offer. On acceptance a practitioner profile is created if needed; Patient access remains.
ID 05	Accept the same offer twice	Nothing is duplicated.
ID 06	Decline one offer; let another expire	Only those offers end. Other offers and memberships remain.
ID 07	Send offers to a new email, an existing account and a staff email	The sender sees exactly the same "Invitation sent" response each time.
ID 08	Offer Consultant to a person declared as Associate Doctor	Offer is sent without the Consultant role; sender sees "some roles need review".

Organization and clinic scope

Test	Action	Expected result
SC 01	Al Noor Owner acts at a clinic created after the grant	Allowed.
SC 02	Organization-wide Practice Manager acts at that new clinic	Allowed.
SC 03	Dubai-only Practice Manager acts at Sharjah	Denied.
SC 04	Dubai-only Practice Manager edits organization defaults	Denied.

Test	Action	Expected result
SC 05	Suspend Sharjah	Provider and RehletShifaa clinic-specific actions at Sharjah are denied; Dubai continues.
SC 06	Suspend Al Noor	All Al Noor clinics denied for everyone; Gulf Heart unaffected.
SC 07	End a clinician's Sharjah link	Sharjah removed from their roles; Dubai and other organizations continue.
SC 08	Send a clinic identifier that does not match the stored record	The stored clinic is used; the mismatch is denied.
SC 09	Sharjah-only user acts at the default clinic (Dubai)	Denied. The default clinic grants nothing.
SC 10	Activate an organization with no active clinic	Refused: no active clinic.
SC 11	Close the default clinic without naming a replacement	Refused, or allowed with acknowledgement and a "default clinic required" warning.

RehletShifaa coverage

Test	Action	Expected result
CC 01	Al Noor Owner looks for or tries to remove Omar's coverage	Not visible in people lists; removal impossible.
CC 02	Omar reviews a Dubai clinician's credentials	Allowed without any provider membership or clinic link.
CC 03	Omar opens a Sharjah or Gulf Heart credential file	Denied.
CC 04	End Dr Sara's Dubai link	Omar's coverage of Dubai is unchanged.
CC 05	System Administrator ends Omar's coverage	No provider membership or clinic link changes.

Roles and eligibility

Test	Action	Expected result
RL 01	Give Consultant to a person declared as Associate Doctor	Refused: level insufficient.
RL 02	Give Consultant before a Consultant credential decision in this organization	Refused, even if another organization has verified the person.
RL 03	Give Associate Doctor with no supervisor	Refused: supervision required.
RL 04	Consultant at one clinic, Associate Doctor at another	Allowed for a Consultant-qualified person with supervision for the Associate work.
RL 05	Consultant Assistant and Consultant at the same clinic	Refused.
RL 06	Remove one role from a person with several	Only that role ends.
RL 07	Give any clinical role to a person with no declared level	Refused.
RL 08	New Consultant role for a level carried over from the old system	Refused until the professional confirms the level.
RL 09	Organization raises the level it is onboarding a person for	Credential level stays unchanged until a new independent decision.

Test	Action	Expected result
RL 10	Dr Sara, as Al Noor Owner, gives herself a Consultant role at Abu Dhabi	Refused: nobody can grant themselves a role.

Separation of duties and administration

Test	Action	Expected result
AD 01	System Administrator opens a patient document	Denied.
AD 02	Compliance Auditor opens a patient case	Denied.
AD 03	System Administrator grants themselves a role	Refused.
AD 04	The only administrator resigns	Blocked until a replacement is active.
AD 05	Provider Operations person decides credentials of a clinician they invited	Refused: independent review required.
AD 06	Give Care Journey Approver to a Care Journey Manager	Refused.
AD 07	Give System Administrator to a Compliance Auditor	Refused.
AD 08	Practice Manager approves a price they wrote	Refused.
AD 09	Owner submits the same transfer twice	One pending nomination only.
AD 10	A System Administrator tries to transfer ownership	Refused.
AD 11	Care Coordinator owning cases changed to Finance Officer	Refused; nothing changes. Succeeds after the cases are transferred.
AD 12	Care Coordinator owning cases promoted to Care Coordination Manager	Allowed; cases stay.

Sign-in and lifecycle

Test	Action	Expected result
AU 01	Owner, administrator or staff member uses a password only	Strong sign-in required before any business action.
AU 02	Accept a manager-role offer without strong sign-in	Asked to use strong sign-in first.
AU 03	A Consultant with no manager role uses a password only	Strong sign-in required (at launch).
AU 04	A patient uses a password only	Patient use is unaffected.
AU 05	Invited staff member signs in with password only	Not activated; no access.
AU 06	Ten first requests arrive at once for an invited staff member	Exactly one activation recorded.
AU 07	Remove one manager role while another qualifying role remains	Strong sign-in still required.
AU 08	Administrator action with a sign-in older than five minutes	Asked to sign in again.

Test	Action	Expected result
AU 09	End one provider membership	Sign-in stays enabled; other memberships continue.

Offboarding

Test	Action	Expected result
OB 01	Complete offboarding for a coordinator who still owns cases	Blocked until the cases are transferred.
OB 02	Assign a new case to a person being offboarded	Refused.
OB 03	Use the person's old session after offboarding completes	No access. History is kept.
OB 04	Offboard the last System Administrator	Blocked.

Context and isolation

Test	Action	Expected result
CX 01	Dr Sara signs in	Exactly three organizations in the selector; one account; no duplicates.
CX 02	Gulf Heart Owner opens an Al Noor page by link	Denied.
CX 03	Gulf Heart manager views Dr Sara's clinician page	Only her Gulf Heart work is shown.
CX 04	Search, notifications and audit for a Gulf Heart manager	No Al Noor or Crescent data appears.
CX 05	"Can they...?" check for Dr Sara at Abu Dhabi	Explains "No authority at Abu Dhabi Hospital" in plain words.

Services, prices, schedules and cases

Test	Action	Expected result
OP 01	Price a consultation where every price level exists	The clinician-at-clinic price is used.
OP 02	Remove the clinician-at-clinic price	The clinic price is used.
OP 03	Price or schedule a service the clinic does not offer	Denied.
OP 04	Overlapping in-person slots at two clinics	The second is refused, also across time zones.
OP 05	Overlapping in-person and virtual slots	The second is refused.
OP 06	In-person appointment step without a clinic	Refused until an eligible active clinic is chosen.
OP 07	Assign a clinician not linked to the case's clinic	Refused.
OP 08	Case with no delivery mode set	Not treated as virtual.
OP 09	Open a case of another organization or clinic	Denied.

Evidence to keep for every test

- Who was signed in and whether strong sign-in was used.
- The organization, clinic, role and scope involved.
- Membership, clinic link, role and eligibility status at the time.
- The response and the plain-language reason for allow or deny.
- The audit entry: who, for whom, organization, clinic, before and after, reason and reference numbers.
- Proof that unrelated organizations, clinics, roles and accounts did not change.

13 Worked example

Meet Dr Sara

Dr Sara Haddad has one sign-in and one practitioner profile. She has declared herself a Consultant. Each organization still gets its own independent credential decision and grants its own local roles.

Worked example: what Dr Sara can do where

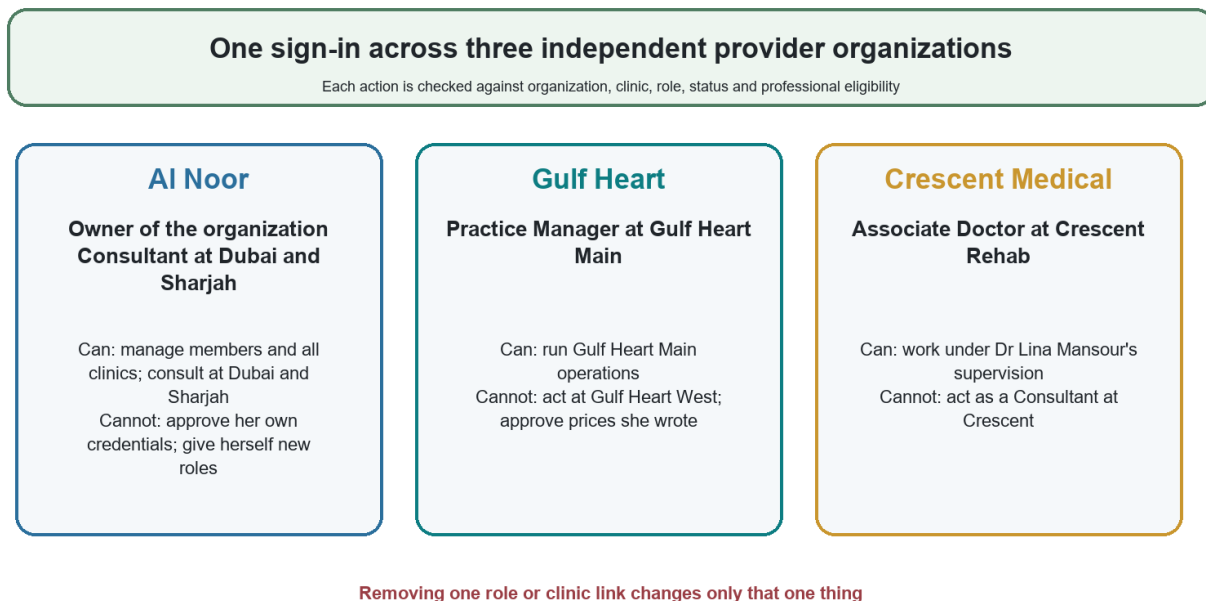


Figure 11 One person, different roles in different places

Organization	Clinics	Roles	Why access works
AI Noor Healthcare Group	Dubai and Sharjah	Organization Owner (whole organization); Consultant at Dubai and Sharjah	Active membership and clinic links; Consultant declaration; Consultant credential decision for AI Noor
Gulf Heart Center	Gulf Heart Main	Practice Manager at Gulf Heart Main	Active membership and clinic link; clinic-limited manager role
Crescent Medical	Crescent Rehab	Associate Doctor	Active membership and clinic link; credential decision for Crescent at Associate level or higher; supervised by Dr Lina Mansour

What Dr Sara may and may not do

Question	Answer	Reason
Can she manage Al Noor's Sharjah Clinic as owner?	Yes	The Owner role covers every Al Noor clinic.
Can she manage Abu Dhabi Hospital as owner?	Yes	Owner reach includes every clinic, even ones she is not linked to.
Can she consult at Abu Dhabi Hospital?	Not yet	Her Consultant role names only Dubai and Sharjah. Another authorized Al Noor manager must add a clinic link and extend the role; she cannot do it for herself.
Can she manage Gulf Heart West?	No	Her manager role names only Gulf Heart Main.
Can she work as a Consultant at Crescent Rehab?	No	Her Crescent role is Associate Doctor, and Crescent has made no Consultant-level decision for her.
Can she approve her own credentials because she is an owner?	No	Self-review is always refused.
Can she make someone a RehletShifaa System Administrator?	No	Provider authority never reaches platform governance.
Can she sign in with a password only when working at Crescent?	No	Strong sign-in follows the person, not the context.

What changes when something ends

1. Gulf Heart removes her Practice Manager role: only that role ends. Al Noor and Crescent are unchanged.
2. Al Noor ends her Sharjah clinic link: she can no longer consult at Sharjah, but as Organization Owner she can still manage Sharjah. Dubai consulting continues.
3. Crescent's supervision relationship ends: her Associate Doctor work at Crescent pauses until supervision is restored. Other organizations are unchanged.
4. Gulf Heart suspends Gulf Heart Main: work at Main stops for everyone there. Al Noor and Crescent are unchanged.
5. RehletShifaa disables her sign-in after a confirmed account compromise: everything stops until the global action is resolved.

Example test script

Step	Action	Expected result
1	Sign in as Dr Sara with strong sign-in	Three organizations in the selector; one account.
2	Choose Al Noor, then Dubai Clinic	Owner and Consultant actions allowed, each by its own rules.
3	Switch to Sharjah Clinic	Consultant work allowed through the Sharjah link.
4	Switch to Abu Dhabi Hospital	Owner management allowed; consulting denied.
5	Switch to Gulf Heart Main	Practice Manager actions allowed; no clinical role there.
6	Open a Gulf Heart West page by link	Denied.
7	Switch to Crescent Rehab	Associate Doctor work allowed while supervision is active.
8	Remove the Gulf Heart manager role; repeat steps 2 and 7	Only Gulf Heart access has ended.

14 Quick reference, glossary and decision status

Role catalogue at a glance

Group	Roles
Platform governance	Platform Account Owner; System Administrator
RehletShifaa internal staff	Care Coordination Manager; Care Coordinator; Travel and Logistics Manager; Travel and Logistics Specialist; Commercial and Finance Manager; Finance Officer; Provider Operations Manager; Provider Operations Specialist; Credentialing Specialist; Patient Identity Reviewer; Care Journey Manager; Care Journey Approver; Compliance Auditor
Provider organization	Organization Owner; Practice Manager; Consultant; Associate Doctor; Consultant Assistant
Patients	Patient; Patient Representative

Glossary

Term	Meaning
Platform Account Owner	Owns the RehletShifaa platform account. Not a provider role.
Organization Owner	Owns one provider organization. Has no platform governance authority.
Provider organization	The legal or contracting provider party, and the boundary between providers.
Clinic or facility	A clinic, branch, hospital, center or virtual-care site inside one organization. Shown as "Clinics & branches".
Default clinic	The one clinic per organization that pre-fills forms. Grants nothing.
Membership	A provider person's relationship with one organization, with its own start and end.
Clinic link (affiliation)	Where a provider member works. Grants nothing by itself.
RehletShifaa coverage	A RehletShifaa employee's assignment to serve an organization or clinics without becoming provider staff.
Role	What a person may do.
Scope	Where the role applies: the organization, selected clinics, managed clinicians, self, or the platform.
Offer	An invitation to join an organization that the person must accept.
Declared level	The professional's own statement: Consultant or Associate Doctor.
Credential decision	The level verified for one organization by an independent RehletShifaa Credentialing Specialist.
Strong sign-in	Sign-in with a second factor such as an authenticator app. Some actions also need it within the last five minutes.
First production release (P1)	The single release that brings all of this model live together.

Before allowing any protected action

1. Is the person's sign-in enabled?

2. For RehletShifaa staff: is the staff record active (or in offboarding handover)?
3. Is the provider membership or RehletShifaa coverage active in this organization?
4. Is the organization active?
5. Is the clinic active, when the record belongs to a clinic?
6. For a provider role limited to clinics: is the person's link to this clinic active?
7. Is the role active, and does its scope cover this record?
8. Was strong sign-in used, and recently enough, when required?
9. For clinical work: are the declared level, credential decision and supervision valid?
10. Are self-approval and separation-of-duty rules respected?

Decision status

The architecture lists fifteen business choices. Two are explicitly confirmed, one is still pending, and the rest stand as adopted defaults.

#	Business choice	Status	What this guide assumes
1	Final English and Arabic names, including clinic terms	Pending native-language review (wording only)	Current English working names
6	Strong sign-in for clinicians	Confirmed	Required at launch
9	Several clinics and several organizations at launch	Confirmed	Yes, in the first production release
2	"Travel and Logistics" as the travel team's name	Adopted default	As named
3	Commercial and Finance as one function	Adopted default	Manager plus Officer
4	Recovery waiting period and request expiry	Adopted default	72 hours each
5	Platform owner may also be an administrator	Adopted default	Only via another administrator's request and approval
7	Extra-responsibility set and conflicts	Adopted default	As in Section 8
8	Second-factor types	Adopted default	Authenticator app required; security keys optional
10	Consultant-qualified person as Associate elsewhere	Adopted default	Allowed with supervision, not at the same clinic
11	RehletShifaa employee as provider member	Adopted default	Refused
12	Offer expiry	Adopted default	14 days
13	Concurrent virtual slots	Adopted default	Not allowed unless the virtual clinic opts in
14	When a case needs a clinic	Adopted default	Before in-person care, travel finalization or location-specific service
15	Clinic-aware routing at launch	Adopted default	Routing by organization; clinic as eligibility filter

Not yet decided (not covered by the architecture)

Question	Why it matters
Who may grant and remove the Organization Owner role, must every organization keep at least one active owner, and how does organization ownership pass to someone else?	Without a rule, an organization could lose its last owner, or ownership could change without a clear approver.

Question	Why it matters
Can one person's membership be paused temporarily in one organization, and can a provider member end their own membership?	The model defines only ending a membership by an authorized manager.

Final rule

A person can have several legitimate relationships without gaining broad or accidental access. RehletShifaa keeps one identity per person, records every organization and clinic relationship separately, checks each role in its exact context, and changes only the relationship that an authorized action names.

Source: Customer Governance Target Architecture (Reviewed), revision 3 (final), decisions D1 to D35 and business choices 1 to 15. Edition 2 corrects Edition 1; the review of Edition 1 is kept alongside this document.